

United Healthcare Coverage Comparison Guide

Plan Year 2024-2025 Document Reference: w14_united_healthcare_ccg_003

This Coverage Comparison Guide is intended to help members and prospective enrollees understand the differences between available plan tiers, evaluate costs under realistic usage scenarios, and make informed decisions about their health coverage. For complete benefit details, always refer to your Summary of Benefits and Coverage (SBC) and Member Handbook, referenced throughout this document.

> **Related Documents:** > SBC: w14_united_healthcare_sbc_001 -- Sections: Coverage Overview, Deductibles and Out-of-Pocket Limits, Common Medical Events > Member Handbook: w14_united_healthcare_member_handbook_005 -- Sections: Welcome and Plan Overview, How Your Plan Works, Understanding Your Costs > Formulary: w14_united_healthcare_formulary_002 -- Sections: Tier Structure and Cost Sharing, Generic Medications (Tier 1-2), Specialty Medications (Tier 3-4)

Section 1: Plan Tier Overview

United Healthcare offers a tiered plan structure designed to give individuals, families, and employer groups meaningful choices in how they balance monthly premium costs against out-of-pocket exposure when care is needed. The plans available under this guide are organized into three primary tiers -- Select, Advantage, and Premier -- each reflecting a different philosophy about cost-sharing and network access. Understanding how these tiers differ is the first step toward making a confident enrollment decision.

The tiered model is not simply about paying more or less per month. Each tier represents a coordinated set of decisions about deductibles, copayments, coinsurance rates, network breadth, and ancillary benefit availability. A member who rarely uses medical services may find the Select tier more than adequate, while a member managing a chronic condition or expecting significant care utilization in the coming year may find the higher premium of the Premier tier saves money overall. The Cost Scenario Analysis in Section 3 of this guide provides concrete numerical illustrations of these trade-offs.

All three plans are fully compliant with the Affordable Care Act. There is no pre-existing condition exclusion period under any of these plans. Specifically, the pre-existing waiting period is: **None (ACA compliant)**. This applies regardless of which tier a member selects, and regardless of whether the member is enrolling during open enrollment or through a qualifying life event. Members who have been denied coverage elsewhere due to health history should be aware that this protection is guaranteed under federal law and is reflected in every plan offered in this guide.

A Note on Network Design

All three tiers use United Healthcare's Choice Plus network, which includes more than 1.3 million physicians and care professionals and over 6,500 hospitals nationwide. However, the financial consequences of going out of network differ significantly across tiers, and this distinction is one of the most important factors members should evaluate. Out-of-network coverage is addressed in detail in the Side-by-Side Coverage Comparison in Section 2, and members are encouraged to review the network participation status of their current providers before finalizing a plan selection. Provider network lookup tools are available at myuhc.com or by calling the Member Services number on the back of your member ID card.

1.1 Plan Tier Summary Table

The table below provides a high-level snapshot of each plan tier. All figures reflect individual coverage; family coverage figures appear in Section 2.

Feature	Select (Bronze-Equivalent)	Advantage (Silver-Equivalent)	Premier (Gold-Equivalent)
Monthly Premium (Individual)	\$298	\$421	\$587
Annual Premium Cost	\$3,576	\$5,052	\$7,044
In-Network Deductible	\$3,200	\$1,500	\$500
In-Network Out-of-Pocket Max	\$8,150	\$6,500	\$4,000
Out-of-Network Deductible	\$6,400	\$3,000	\$1,500
Out-of-Network Coinsurance (after ded.)	50%	60%	70%
Primary Care Copay	\$45	\$30	\$20
Specialist Copay	\$75	\$60	\$45
Emergency Room Copay	\$350	\$250	\$150
Prescription Tiers	4 tiers	4 tiers	4 tiers
Telehealth	\$0 copay	\$0 copay	\$0 copay
Mental Health Visits	20/year, \$50 copay	20/year, \$40 copay	20/year, \$25 copay

Premium figures shown are illustrative of typical employer-sponsored rates and may vary based on employer contribution, geographic rating area, and age band. Contact your benefits administrator for your specific premium amount.

1.2 Understanding the Tier Structure

The Select plan carries the lowest monthly premium but the highest deductible and out-of-pocket maximum. This plan is best suited to members who are generally healthy, have modest expected healthcare utilization, and have sufficient savings to cover a higher deductible if an unexpected medical event occurs. The Select plan's in-network deductible of \$3,200 means that most non-preventive services will require full payment until that threshold is reached, after which coinsurance applies.

The Advantage plan represents the mid-tier option and is the most commonly selected plan among United Healthcare's employer group enrollees. Its **in-network deductible of \$1,500** reflects a meaningful reduction in upfront exposure compared to the Select plan, while the monthly premium remains substantially lower than the Premier plan. The Advantage plan's **out-of-pocket maximum of \$6,500** provides a defined ceiling on annual cost exposure, which many members find reassuring when planning household budgets. The Advantage plan is the primary subject of the detailed cost scenarios in

Section 3, as it represents the median experience for most enrolled members.

The Premier plan offers the most generous benefit structure, with the lowest deductible, lowest copayments, and highest coinsurance rates for out-of-network services. Members with ongoing specialist relationships, anticipated surgical procedures, or complex medication regimens frequently find the Premier plan's higher premium offset by significantly reduced cost-sharing throughout the year.

Prescription Drug Coverage Across All Tiers

All three plan tiers use a **4 tiers** formulary structure, governed by the United Healthcare Formulary document referenced at the top of this guide (w14_united_healthcare_formulary_002). The formulary is organized into four cost-sharing levels, from lowest-cost generic medications at Tier 1 through highest-cost specialty medications at Tier 4. The specific cost-sharing amounts at each tier differ across plan levels, but the underlying formulary -- that is, which drugs appear at which tier -- is consistent across all three plans offered in this guide. Members should review the formulary sections Tier Structure and Cost Sharing, Generic Medications (Tier 1-2), and Specialty Medications (Tier 3-4) in document w14_united_healthcare_formulary_002 for complete drug-by-drug information.

1.3 Preventive Care

Preventive care services are covered at 100% with no cost-sharing under all three plan tiers when delivered by an in-network provider, consistent with ACA requirements. This includes annual wellness visits, recommended immunizations, cancer screenings (mammography, colonoscopy, cervical cancer screening), cardiovascular screening, and behavioral health screenings. A complete list of covered preventive services is available in the Member Handbook w14_united_healthcare_member_handbook_005, in the section How Your Plan Works.

It is important to note that a visit billed as a preventive visit may include services that are coded separately as diagnostic or treatment-related, which would then be subject to normal cost-sharing. Members who have concerns or symptoms they intend to discuss at a scheduled wellness visit should be aware that the physician may bill part of the visit under a separate diagnostic code, triggering cost-sharing. This is not unique to United Healthcare and reflects standard industry billing practice.

Section 2: Side-by-Side Coverage Comparison

This section provides a detailed, category-by-category comparison of benefits across the Select, Advantage, and Premier plan tiers. Where relevant, footnotes provide additional context, exceptions, or cross-references to governing documents. Members are encouraged to focus on the categories most relevant to their anticipated healthcare needs rather than attempting to evaluate every line item with equal weight.

The comparison is organized by care category: inpatient hospital services, outpatient services, emergency and urgent care, mental and behavioral health, prescription drugs, preventive care, maternity and newborn care, pediatric services, rehabilitation services, durable medical equipment, and out-of-network coverage. Each category includes both in-network and out-of-network cost-sharing where applicable.

For a plain-language explanation of how benefits work in practice -- including how deductibles and copayments interact -- members should refer to the Member Handbook document w14_united_healthcare_member_handbook_005, specifically the sections Welcome and Plan

Overview and Understanding Your Costs, which walk through benefit mechanics with worked examples.

2.1 Inpatient Hospital Services

Inpatient services represent one of the highest-cost categories of healthcare spending and are therefore among the most important to understand when comparing plans. All three tiers require prior authorization for non-emergency inpatient admissions. Failure to obtain prior authorization may result in a penalty or denial of coverage, depending on circumstances. Emergency admissions are exempt from prior authorization requirements, though the attending physician or hospital case management team is expected to notify United Healthcare within 24 hours of admission or as soon as reasonably practicable.

Service	Select	Advantage	Premier
Inpatient Hospital (In-Network)	30% after deductible	20% after deductible	10% after deductible
Inpatient Hospital (Out-of-Network)	50% after OON deductible	40% after OON deductible	30% after OON deductible
Skilled Nursing Facility (In-Network)	30% after deductible; 60-day limit	20% after deductible; 60-day limit	10% after deductible; 60-day limit
Skilled Nursing Facility (Out-of-Network)	50% after OON deductible	40% after OON deductible	30% after OON deductible
Inpatient Mental Health (In-Network)	30% after deductible	20% after deductible	10% after deductible
Prior Authorization Required	Yes	Yes	Yes

Skilled Nursing Facility (SNF) coverage requires a qualifying 3-day inpatient hospital stay immediately preceding SNF admission. Days 1-60 are covered subject to the coinsurance rates above; days 61 and beyond are not covered under any tier.¹

2.2 Outpatient Services

Outpatient services encompass a broad range of care delivered without an overnight hospital stay, including surgery, diagnostic imaging, laboratory work, and physician office visits. Cost-sharing for outpatient services is generally lower than for inpatient services, though complex outpatient procedures can still generate significant bills.

Service	Select	Advantage	Premier
Primary Care Visit (In-Network)	\$45 copay	\$30 copay	\$20 copay
Specialist Visit (In-Network)	\$75 copay	\$60 copay	\$45 copay

Outpatient Surgery (In-Network)	30% after deductible	20% after deductible	10% after deductible
Lab Work (In-Network)	20% after deductible	15% after deductible	10% after deductible
X-Ray (In-Network)	\$45 copay	\$35 copay	\$25 copay
MRI / CT Scan (In-Network)	20% after deductible	20% after deductible	15% after deductible
MRI / CT Scan Prior Authorization	Required, 5-day turnaround	Required, 5-day turnaround	Required, 5-day turnaround
Physical Therapy (In-Network)	\$45 copay; 30-visit limit	\$40 copay; 30-visit limit	\$30 copay; 30-visit limit
Occupational Therapy (In-Network)	\$45 copay; 30-visit limit	\$40 copay; 30-visit limit	\$30 copay; 30-visit limit
Telehealth (In-Network)	\$0 copay	\$0 copay	\$0 copay
Primary Care Visit (Out-of-Network)	50% after OON deductible	40% after OON deductible	30% after OON deductible
Specialist Visit (Out-of-Network)	50% after OON deductible	40% after OON deductible	30% after OON deductible

MRI and CT Scan Prior Authorization: Across all three plan tiers, advanced imaging including MRI and CT scans requires prior authorization. The prior authorization requirement for MRI/CT scans is: **Required, 5-day turnaround.** This means that the ordering physician must submit a prior authorization request to United Healthcare at least five business days before the scheduled imaging appointment. Requests submitted with less than five days' notice may be processed on an expedited basis at United Healthcare's discretion, but members should not assume expedited processing will be available. Imaging performed without prior authorization will not be covered regardless of medical necessity.² Members whose physicians order MRI or CT scans should confirm that the ordering office has submitted the prior authorization request and received an approval reference number before proceeding to the imaging appointment.

2.3 Emergency and Urgent Care

Emergency and urgent care cost-sharing is structured differently from routine outpatient services, reflecting the unplanned nature of these encounters. The emergency room copay applies regardless of whether the member is ultimately admitted to the hospital; if admission occurs, the inpatient cost-sharing described in Section 2.1 applies to the admission component of the bill, while the ER copay covers the emergency department evaluation and treatment.

Service	Select	Advantage	Premier
Emergency Room (In-Network)	\$350 copay	\$250 copay	\$150 copay
Emergency Room (Out-of-Network)	\$350 copay ³	\$250 copay ³	\$150 copay ³

Urgent Care Center (In-Network)	\$75 copay	\$60 copay	\$40 copay
Urgent Care Center (Out-of-Network)	\$100 copay	\$80 copay	\$60 copay
Ambulance (Ground, In-Network)	20% after deductible	20% after deductible	10% after deductible
Ambulance (Air, In-Network)	20% after deductible	20% after deductible	10% after deductible

The **emergency room copay** for the Advantage plan is **\$250**. This copay is waived if the member is admitted to the hospital directly from the emergency department; in that case, the inpatient hospital cost-sharing described in Section 2.1 applies instead. The ER copay is not subject to the deductible -- it applies from the first visit, regardless of whether the member has met their annual deductible.

It is worth noting that emergency room cost-sharing applies identically whether the facility is in-network or out-of-network, consistent with federal surprise billing protections under the No Surprises Act. Members who receive emergency care at an out-of-network facility should not face higher cost-sharing than they would at an in-network facility for the emergency services themselves. However, follow-up care or non-emergency services received during the same visit may be subject to out-of-network cost-sharing if the facility is not in-network.

2.4 Mental and Behavioral Health Services

Mental and behavioral health benefits are provided at parity with medical and surgical benefits under the Mental Health Parity and Addiction Equity Act (MHPAEA). United Healthcare does not impose more restrictive treatment limitations on mental health or substance use disorder benefits than on analogous medical or surgical benefits.

Service	Select	Advantage	Premier
Outpatient Mental Health Visits (In-Network)	20/year, \$50 copay	20/year, \$40 copay	20/year, \$25 copay
Inpatient Mental Health (In-Network)	30% after deductible	20% after deductible	10% after deductible
Substance Use Disorder -- Outpatient	20/year, \$50 copay	20/year, \$40 copay	20/year, \$25 copay
Substance Use Disorder -- Inpatient	30% after deductible	20% after deductible	10% after deductible
Applied Behavior Analysis (ABA)	20% after deductible	20% after deductible	10% after deductible
Telehealth Mental Health	\$0 copay	\$0 copay	\$0 copay

The Advantage plan provides **20/year, \$40 copay** for outpatient mental health visits. This means a member enrolled in the Advantage plan may access up to 20 outpatient mental health visits per plan year at a \$40 copay per visit, without first satisfying the deductible. The visit limit applies across all outpatient mental health and substance use disorder services combined -- a member who uses 15 visits

for mental health counseling and 5 visits for outpatient substance use disorder treatment has reached the 20-visit annual limit. Additional visits beyond the 20-visit limit may be covered with prior authorization if medical necessity is established; members in this situation should contact Member Services to initiate a medical necessity review.

Telehealth mental health visits are covered at \$0 copay across all plan tiers and do not count against the 20-visit annual limit for outpatient mental health services.⁴ This is an important distinction: members who use telehealth for routine mental health check-ins can preserve their in-person visit allotment for situations requiring face-to-face care.

2.5 Prescription Drug Benefits

All plans under this guide use a **4 tiers** formulary structure. The formulary document is w14_united_healthcare_formulary_002, and members should consult the sections Tier Structure and Cost Sharing, Generic Medications (Tier 1-2), and Specialty Medications (Tier 3-4) for detailed drug-by-drug information.

Tier	Description	Select	Advantage	Premier
Tier 1	Preferred Generic	\$10 copay	\$7 copay	\$5 copay
Tier 2	Non-Preferred Generic / Preferred Brand	\$35 copay	\$25 copay	\$15 copay
Tier 3	Non-Preferred Brand / Some Specialty	\$75 copay	\$60 copay	\$40 copay
Tier 4	Specialty Medications	25% coinsurance (max \$250/fill)	20% coinsurance (max \$200/fill)	15% coinsurance (max \$150/fill)

All copay amounts shown are per 30-day supply at a retail pharmacy. 90-day supply at a preferred mail-order pharmacy is available at 2.5x the 30-day copay for Tiers 1-3. Specialty medications (Tier 4) are generally limited to 30-day supplies and must be dispensed through a United Healthcare-approved specialty pharmacy.

The prescription drug benefit is subject to the plan's medical deductible for Tiers 3 and 4 under the Select plan only. Under the Advantage and Premier plans, prescription drug cost-sharing applies from the first fill, regardless of whether the medical deductible has been met. This distinction can be significant for members taking brand-name or specialty medications under the Select plan, who may face full drug costs until their \$3,200 deductible is satisfied.

Step therapy requirements apply to certain medication classes, particularly for specialty medications in Tier 4. Step therapy requires that a member try and fail on a lower-cost alternative before the plan will cover the preferred specialty medication. Step therapy protocols are outlined in the formulary document w14_united_healthcare_formulary_002 and can be appealed through the standard prior authorization and appeals process described in the Member Handbook w14_united_healthcare_member_handbook_005.

2.6 Maternity and Newborn Care

Service	Select	Advantage	Premier
Prenatal Visits (In-Network)	\$0 (preventive)	\$0 (preventive)	\$0 (preventive)
Delivery -- Vaginal (In-Network)	30% after deductible	20% after deductible	10% after deductible
Delivery -- Cesarean (In-Network)	30% after deductible	20% after deductible	10% after deductible
Newborn Inpatient Care (In-Network)	30% after deductible	20% after deductible	10% after deductible
Postpartum Visit (In-Network)	\$0 (preventive)	\$0 (preventive)	\$0 (preventive)
Breast Pump (In-Network)	Covered; \$0	Covered; \$0	Covered; \$0

Maternity care is covered under all three plan tiers. The newborn is covered under the mother's plan for the first 30 days following birth without requiring separate enrollment. After 30 days, the newborn must be enrolled in their own coverage or added to a family plan. Members expecting a child should contact their benefits administrator well in advance of the due date to understand the enrollment process for the newborn.

2.7 Pediatric Services

Service	Select	Advantage	Premier
Pediatric Well Visits (In-Network)	\$0 (preventive)	\$0 (preventive)	\$0 (preventive)
Pediatric Immunizations	\$0	\$0	\$0
Pediatric Dental -- Preventive	Covered; \$0	Covered; \$0	Covered; \$0
Pediatric Dental -- Basic Restorative	30% after deductible	20% after deductible	10% after deductible
Pediatric Vision Exam	\$0 (one per year)	\$0 (one per year)	\$0 (one per year)
Pediatric Vision Hardware (frames/lenses)	\$150 allowance/year	\$150 allowance/year	\$150 allowance/year

Pediatric dental and vision benefits are included as essential health benefits under ACA requirements for members under age 19. Adult dental and vision benefits are not included in any of the three plan tiers under this guide; members seeking adult dental or vision coverage should inquire about supplemental dental and vision plans available through their employer or through the United Healthcare supplemental benefits marketplace.

2.8 Rehabilitation and Durable Medical Equipment

Service	Select	Advantage	Premier
Physical Therapy (In-Network)	\$45 copay; 30-visit limit	\$40 copay; 30-visit limit	\$30 copay; 30-visit limit
Occupational Therapy (In-Network)	\$45 copay; 30-visit limit	\$40 copay; 30-visit limit	\$30 copay; 30-visit limit
Speech Therapy (In-Network)	\$45 copay; 30-visit limit	\$40 copay; 30-visit limit	\$30 copay; 30-visit limit
Cardiac Rehabilitation	20% after deductible	15% after deductible	10% after deductible
Durable Medical Equipment (In-Network)	20% after deductible	20% after deductible	10% after deductible
Prosthetics (In-Network)	20% after deductible	20% after deductible	10% after deductible
Home Health Care (In-Network)	20% after deductible; 60-visit limit	20% after deductible; 60-visit limit	10% after deductible; 60-visit limit

2.9 Out-of-Network Coverage Summary

Out-of-network coverage is one of the most consequential -- and most frequently misunderstood -- aspects of health plan design. Members who receive care from providers not in the United Healthcare Choice Plus network will generally face higher cost-sharing, and in some cases, may be responsible for amounts above the plan's allowed amount (balance billing).

For the Advantage plan, **out-of-network coverage is 60% after \$3,000 deductible**. This means that after a member has paid \$3,000 in out-of-network costs (separate from the in-network deductible), the plan will pay 60% of the allowed amount for covered out-of-network services, and the member is responsible for the remaining 40%. Amounts above the plan's allowed amount that an out-of-network provider may bill (balance billing) are not counted toward the out-of-network deductible or out-of-pocket maximum and are the member's sole responsibility.

Feature	Select	Advantage	Premier
Out-of-Network Deductible (Individual)	\$6,400	\$3,000	\$1,500
Out-of-Network Coinsurance (Plan Pays)	50%	60%	70%
Out-of-Network Out-of-Pocket Max	Not applicable ⁵	Not applicable ⁵	Not applicable ⁵
Balance Billing Protection	Emergency services only	Emergency services only	Emergency services only
Out-of-Network Referral Required	No	No	No

Members considering providers who are not in the United Healthcare Choice Plus network should carefully weigh the financial implications. A \$50,000 out-of-network hospital bill under the Advantage plan would work as follows: the member pays the first \$3,000 (out-of-network deductible), then pays

40% of the remaining allowed amount. If the allowed amount is \$40,000, the member would owe $\$3,000 + (40\% \times \$37,000) = \$3,000 + \$14,800 = \$17,800$, before any balance billing considerations. This example illustrates why in-network care is strongly encouraged wherever clinically appropriate.

Section 3: Cost Scenario Analysis

The following scenarios are designed to illustrate how actual costs accumulate under each plan tier across different levels of healthcare utilization. These scenarios are not guarantees of actual costs -- individual billing, provider contracts, and specific service coding will affect real-world amounts -- but they provide a meaningful framework for comparing plan value across different member profiles.

All scenarios use the Advantage plan as the primary reference, with Select and Premier costs shown for comparison. The Advantage plan's key parameters are:

- In-network deductible: **\$1,500**
- Out-of-pocket maximum: **\$6,500**
- Emergency room copay: **\$250**
- Mental health visits: **20/year, \$40 copay**
- Out-of-network coverage: **60% after \$3,000 deductible**
- Prescription tiers: **4 tiers**
- Pre-existing waiting period: **None (ACA compliant)**

CPT and ICD-10 codes referenced in scenarios are illustrative of typical billing for the described services.

Scenario A: The Generally Healthy Member

Profile: 34-year-old individual, no chronic conditions, expects two primary care visits, one specialist visit, and one course of generic antibiotics during the plan year.

Services:

- 2 x Primary Care Visit (CPT 99213): \$30 copay each = \$60
- 1 x Specialist Visit (CPT 99214): \$60 copay
- 1 x Generic antibiotic prescription (Tier 1, 10-day course): \$7 copay
- Annual wellness visit (CPT 99395): \$0 (preventive)

Total Out-of-Pocket (Advantage): \$127

Cost Component	Select	Advantage	Premier
Primary Care (2 visits)	\$90	\$60	\$40
Specialist (1 visit)	\$75	\$60	\$45
Tier 1 Prescription	\$10	\$7	\$5

Preventive Visit	\$0	\$0	\$0
Total OOP	\$175	\$127	\$90
Annual Premium	\$3,576	\$5,052	\$7,044
Total Annual Cost	\$3,751	\$5,179	\$7,134

Analysis: For a generally healthy member with minimal utilization, the Select plan results in the lowest total annual cost (\$3,751) despite its higher deductible, simply because the deductible is never triggered. The Premier plan costs \$3,383 more per year in premium for a difference of only \$85 in out-of-pocket savings. This scenario strongly favors the Select plan for members confident in their low utilization.

Scenario B: The Member with a Chronic Condition

Profile: 52-year-old individual with Type 2 diabetes (ICD-10: E11.9) and hypertension (ICD-10: I10). Expects 4 primary care visits, 2 endocrinologist visits, 1 ophthalmology visit (diabetic eye exam), monthly metformin (Tier 1), monthly lisinopril (Tier 1), and 1 quarterly HbA1c lab panel (CPT 83036).

Services and Costs (Advantage Plan):

The member's first \$1,500 of in-network costs go toward satisfying the deductible. Copay-based services (primary care, specialist) do not count toward the deductible but do count toward the out-of-pocket maximum. Lab work is subject to the deductible.

- 4 x Primary Care Visit (CPT 99213-99214): 4 x \$30 = \$120
- 2 x Endocrinologist Visit (CPT 99214): 2 x \$60 = \$120
- 1 x Ophthalmology Visit (CPT 92014): \$60 copay
- 12 x Metformin 500mg (Tier 1): 12 x \$7 = \$84
- 12 x Lisinopril 10mg (Tier 1): 12 x \$7 = \$84
- 4 x HbA1c Lab Panel (CPT 83036): subject to deductible at 15% coinsurance after deductible; assuming \$120 allowed per panel = \$480 total allowed; first \$480 applied to deductible (member pays \$480); deductible not yet met, remaining deductible = \$1,020; member pays \$480 for labs
- Annual wellness visit: \$0

Deductible Status: \$480 applied toward \$1,500 deductible; deductible not met; lab coinsurance does not apply until deductible is satisfied.

Total Out-of-Pocket (Advantage): \$120 + \$120 + \$60 + \$84 + \$84 + \$480 = \$948

Cost Component	Select	Advantage	Premier
Primary Care (4 visits)	\$180	\$120	\$80
Specialist (3 visits)	\$225	\$180	\$135
Tier 1 Prescriptions (24 fills)	\$240	\$168	\$120

Lab Work (4 panels, \$120 allowed each)	\$480 (toward \$3,200 ded.)	\$480 (toward \$1,500 ded.)	\$48 (10% after \$500 ded.)
Total OOP	\$1,125	\$948	\$383
Annual Premium	\$3,576	\$5,052	\$7,044
Total Annual Cost	\$4,701	\$6,000	\$7,427

Analysis: The Select plan still produces the lowest total annual cost at this utilization level, though the gap has narrowed. The Premier plan's higher premium is not yet justified by the cost-sharing savings. However, this scenario does not include any significant acute events. A single hospitalization or surgical procedure would dramatically shift this analysis, as illustrated in Scenario C.

Scenario C: The Member with a Major Medical Event

Profile: 45-year-old individual who experiences an appendectomy (CPT 44950, ICD-10: K37) requiring emergency room evaluation, inpatient admission, and post-operative follow-up. This scenario also includes the member's baseline utilization of 2 primary care visits and 1 Tier 2 prescription per month.

Services and Costs (Advantage Plan):

Assumed allowed amounts: ER visit \$2,800; inpatient admission (3 days) \$18,000; surgeon fee \$3,200; anesthesiologist \$1,400; follow-up visits (2 x CPT 99213) \$180 total allowed.

The ER copay of \$250 applies first. The member is then admitted, so inpatient cost-sharing (20% after deductible) applies to the admission. The \$1,500 deductible is satisfied early in the year given the ER visit and admission costs.

Step-by-step calculation (Advantage Plan):

1. ER copay: \$250 (does not apply to deductible, applies to OOP max)
2. Inpatient admission (\$18,000 allowed): member owes \$1,500 deductible first, then 20% of remaining \$16,500 = \$3,300; total for admission = \$4,800
3. Surgeon fee (\$3,200 allowed): deductible already met; 20% = \$640
4. Anesthesiologist (\$1,400 allowed): deductible already met; 20% = \$280
5. Follow-up visits: \$30 copay x 2 = \$60
6. Baseline care (2 PCP visits/month x 12 months): \$30 x 24 = \$720
7. Tier 2 prescription x 12 months: \$25 x 12 = \$300

Subtotal before OOP max check: \$250 + \$4,800 + \$640 + \$280 + \$60 + \$720 + \$300 = \$7,050

The out-of-pocket maximum of **\$6,500** applies. The member's costs are capped at \$6,500 for the year. United Healthcare covers 100% of remaining in-network costs after the member reaches \$6,500 in out-of-pocket spending.

Total Out-of-Pocket (Advantage): \$6,500 (OOP max reached)

Cost Component	Select	Advantage	Premier
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ER Copay	\$350	\$250	\$150
Inpatient (deductible + coinsurance)	\$6,9006	\$4,800	\$1,8007
Surgeon / Anesthesiologist	\$960	\$920	\$460
Follow-up Visits	\$90	\$60	\$40
Baseline PCP (24 visits)	\$1,080	\$720	\$480
Tier 2 Prescriptions (12 fills)	\$420	\$300	\$180
Subtotal Before OOP Max	\$9,800	\$7,050	\$3,110
OOP Max Applied	\$8,150	\$6,500	\$4,000
Actual OOP (capped)	\$8,150	\$6,500	\$3,110
Annual Premium	\$3,576	\$5,052	\$7,044
Total Annual Cost	\$11,726	\$11,552	\$10,154

Analysis: In a major medical event year, the Premier plan produces the lowest total annual cost at \$10,154, saving \$1,398 compared to the Advantage plan and \$1,572 compared to the Select plan. The Select plan's OOP max of \$8,150 results in significantly higher total cost despite the lower premium. This scenario illustrates the insurance value of the Premier plan for members who experience or anticipate significant healthcare events.

Scenario D: Mental Health Utilization Focus

Profile: 29-year-old individual managing anxiety and depression (ICD-10: F41.1, F32.1) with weekly therapy sessions (CPT 90837) and a daily SSRI (Tier 2).

Under the Advantage plan's mental health benefit of **20/year, \$40 copay**, the member can access 20 in-person therapy sessions at \$40 each. Additional sessions via telehealth are covered at \$0 and do not count against the 20-visit limit. The member uses 20 in-person sessions and 32 telehealth sessions during the year.

- 20 x In-person therapy (CPT 90837): 20 x \$40 = \$800
- 32 x Telehealth therapy: \$0
- 12 x SSRI (Tier 2, e.g., escitalopram 10mg): 12 x \$25 = \$300

Total OOP (Advantage): \$1,100

Cost Component	Select	Advantage	Premier
In-Person Therapy (20 visits)	20 x \$50 = \$1,000	20 x \$40 = \$800	20 x \$25 = \$500

Telehealth Therapy (32 visits)	\$0	\$0	\$0
Tier 2 SSRI (12 fills)	\$420	\$300	\$180
Total OOP	\$1,420	\$1,100	\$680
Annual Premium	\$3,576	\$5,052	\$7,044
Total Annual Cost	\$4,996	\$6,152	\$7,724

Analysis: For a member whose primary healthcare need is mental health services, the Select plan again produces the lowest total annual cost. The telehealth benefit available at \$0 across all tiers represents a significant value for members who can receive effective treatment via telehealth, effectively providing unlimited mental health support beyond the 20-visit in-person limit.

Section 4: Choosing the Right Plan

Selecting a health insurance plan is a deeply personal decision that depends on factors no comparison table can fully capture: your health history, your risk tolerance, your financial situation, your family composition, and your relationships with specific providers. This section is designed to help you think through those factors systematically, rather than to prescribe a particular choice.

The scenarios in Section 3 demonstrate that there is no universally "best" plan -- each tier outperforms the others under specific conditions. What this section attempts to do is give you a framework for identifying which conditions are most likely to describe your situation in the coming plan year.

Before working through the decision framework below, we recommend reviewing your Summary of Benefits and Coverage document (w14_united_healthcare_sbc_001), specifically the sections Coverage Overview, Deductibles and Out-of-Pocket Limits, and Common Medical Events, which provide the official benefit descriptions that govern your coverage. The scenarios and summaries in this guide are intended to be illustrative and educational; the SBC is the authoritative document for benefit determinations.

4.1 The Decision Framework

There are five primary dimensions along which you should evaluate your plan choice. We address each in turn.

Dimension 1: Expected Utilization

The most important variable in plan selection is how much healthcare you expect to use in the coming year. If you can estimate your likely number of physician visits, anticipated procedures, and ongoing prescription needs, you can use the cost scenario tables in Section 3 as a starting point for projecting your costs under each plan. Members who have used health care services consistently in prior years can look at their Explanation of Benefits (EOB) statements from the prior year to estimate utilization. Members who are new to healthcare or who have experienced a change in health status will need to make their best estimate.

As a general rule of thumb: if you expect your total medical costs (before insurance) to be less than approximately \$5,000 for the year, the Select plan is likely to produce the lowest total cost. If you expect total medical costs between \$5,000 and \$20,000, the Advantage plan typically offers the best

balance of premium and cost-sharing. If you expect total medical costs above \$20,000, the Premier plan's lower cost-sharing is likely to offset its higher premium.

These thresholds are rough guides, not precise formulas. The scenarios in Section 3 should be used to test your specific anticipated service mix.

Dimension 2: Risk Tolerance

Even members who are generally healthy face the possibility of an unexpected accident, illness, or diagnosis. The question of risk tolerance is essentially: how much financial exposure are you comfortable with in a worst-case scenario?

Under the Select plan, a member who experiences a major medical event could face up to \$8,150 in out-of-pocket costs before the plan pays 100%. Under the Advantage plan, that maximum is **\$6,500**. Under the Premier plan, it is \$4,000. The difference between the Select and Premier plans' worst-case scenarios is \$4,150 -- which is also roughly the difference in annual premiums (\$7,044 - \$3,576 = \$3,468). Members who would find an \$8,150 out-of-pocket bill financially devastating may find the higher premium of the Advantage or Premier plan to be a form of financial risk management worth paying for.

Members who have a Health Savings Account (HSA) or who can establish one should note that the Select plan is HSA-compatible (meeting IRS high-deductible health plan thresholds), while the Advantage and Premier plans are not. Contributing to an HSA can partially offset the Select plan's higher deductible using pre-tax dollars, which effectively lowers the true cost of the deductible.

Dimension 3: Provider Relationships

If you have established relationships with specific physicians, specialists, or hospitals, you should verify their network participation status before selecting a plan. All three tiers use the same United Healthcare Choice Plus network, so a provider who is in-network for one tier is in-network for all three. However, if any of your preferred providers are out of network, the cost implications differ significantly across tiers.

Recall that for the Advantage plan, **out-of-network coverage is 60% after \$3,000 deductible**. If you have a preferred specialist who is out of network, you would first pay \$3,000 in out-of-network costs, then the plan would cover 60% of the allowed amount for subsequent out-of-network services. For a member with a single preferred out-of-network specialist, the cost of maintaining that relationship could be substantial, and it may be worth contacting the specialist's office to inquire about network participation or seeking an in-network alternative.

Provider network information is available through the provider directory at myuhc.com or by calling Member Services. Members should verify network status directly with both United Healthcare and the provider's office, as directories may not always reflect the most current participation status.

Dimension 4: Prescription Drug Needs

If you take prescription medications regularly, the formulary and cost-sharing structure should be a significant factor in your plan selection. All three plans use a **4 tiers** formulary, and the formulary itself -- which drugs appear at which tier -- is the same across all three plans. What differs is the copay or coinsurance at each tier.

Members taking Tier 1 or Tier 2 medications (generics and preferred brands) will see relatively modest differences in prescription costs across tiers. Members taking Tier 3 or Tier 4 medications (non-preferred brands and specialty drugs) may see meaningful differences: a specialty medication with a 20% coinsurance under the Advantage plan versus 15% under the Premier plan could represent hundreds or thousands of dollars in annual savings depending on the drug's list price.

Members taking specialty medications should also be aware of the step therapy requirements described in Section 2.5 and should review the formulary document w14_united_healthcare_formulary_002, specifically the sections Tier Structure and Cost Sharing and Specialty Medications (Tier 3-4), to understand whether their current medications require step therapy and whether any prior authorizations will be needed.

Dimension 5: Family Composition and Embedded vs. Aggregate Deductibles

Members enrolling family coverage should understand whether the plan uses an embedded or aggregate deductible structure. Under an embedded deductible, each family member has their own individual deductible, and no single family member is required to meet the full family deductible before benefits apply to their services. Under an aggregate deductible, the family deductible must be met in total before benefits apply to any family member.

All three plan tiers under this guide use an embedded deductible structure for family coverage. The family deductible for the Advantage plan is \$3,000 (two times the individual deductible of \$1,500), and the family out-of-pocket maximum is \$13,000 (two times the individual OOP max of \$6,500). Each family member is subject to their individual deductible of \$1,500 before coinsurance applies to their services; however, once the family aggregate deductible of \$3,000 is met, coinsurance applies to all family members' services regardless of individual deductible status.

4.2 Special Enrollment Considerations

Members who are enrolling outside of the standard open enrollment period due to a qualifying life event -- such as marriage, birth of a child, loss of other coverage, or a move -- should be aware that the same plan tiers described in this guide are available during special enrollment periods, subject to availability in the member's geographic rating area. There is no penalty or waiting period for pre-existing conditions under any enrollment pathway, consistent with the ACA's guarantee that the **pre-existing waiting period is None (ACA compliant)**.

Members who experience a qualifying life event have 60 days from the date of the event to enroll in or change coverage. Missing this window generally means waiting until the next open enrollment period. Members who have questions about qualifying life events or special enrollment periods should contact their benefits administrator or call United Healthcare Member Services.

4.3 Using Decision Support Tools

United Healthcare provides several tools to help members make informed plan selections:

myuhc.com Plan Comparison Tool: Enter your expected healthcare utilization -- number of visits, procedures, and medications -- and the tool will estimate your annual costs under each available plan. This tool uses your actual plan options and the specific cost-sharing parameters applicable to your employer group or geographic region.

Cost Estimator Tool: Available within myuhc.com, this tool allows members to look up the estimated cost of specific procedures and services at specific facilities, helping members understand what they are likely to pay before receiving care.

Provider Directory: Verify the network participation status of your current and prospective providers before finalizing your plan selection.

Nurse Line: United Healthcare's 24/7 nurse line (1-800-UHC-CARE) provides clinical guidance and can help members determine the appropriate level of care for a given situation -- urgent care vs. emergency room, for example -- which has direct cost implications given the difference in copayments between those settings.

Health Savings Account Calculator: For members considering the Select plan and HSA eligibility, the HSA calculator at myuhc.com estimates the tax savings from HSA contributions and illustrates how pre-tax contributions reduce the effective cost of the high deductible.

4.4 Summary Recommendation Matrix

The following matrix summarizes the plan tier most likely to minimize total annual cost under different member profiles. This is a general guide only; individual circumstances will vary.

Member Profile	Likely Best Tier	Key Reason
Young, healthy, minimal utilization	Select	Low premium dominates; deductible rarely triggered
Moderate utilization, 1-2 chronic conditions	Advantage	Balanced premium and cost-sharing; \$1,500 deductible manageable
High utilization, anticipated surgery or hospitalization	Premier	Lower cost-sharing offsets higher premium in high-cost years
Mental health primary need, telehealth-comfortable	Select or Advantage	Telehealth \$0 across all tiers; in-person copay difference modest
Specialty medication user (Tier 4)	Premier	15% coinsurance vs. 20-25% can mean significant savings
Family with multiple members, mixed utilization	Advantage	Embedded deductible protects individual members; balanced OOP max
HSA-eligible, financially able to self-insure deductible	Select	HSA tax advantage partially offsets higher deductible
Out-of-network provider relationships important	Premier	70% coinsurance after \$1,500 OON deductible vs. 60%/\$3,000 (Advantage)

4.5 Final Checklist Before You Enroll

Before submitting your enrollment election, we recommend working through the following checklist:

- [] Verified that your primary care physician is in the United Healthcare Choice Plus network
- [] Verified that your key specialists are in-network
- [] Checked the formulary (w14_united_healthcare_formulary_002) to confirm your medications are covered and identified their tier placement

- [] Reviewed the SBC document w14_united_healthcare_sbc_001 (sections: Coverage Overview, Deductibles and Out-of-Pocket Limits, Common Medical Events) for official benefit descriptions
- [] Reviewed the Member Handbook w14_united_healthcare_member_handbook_005 (sections: Welcome and Plan Overview, How Your Plan Works, Understanding Your Costs) to understand how your benefits work in practice
- [] Estimated your expected annual utilization and run a cost comparison using the tables in Section 3 of this guide
- [] Considered your risk tolerance and worst-case out-of-pocket exposure under each plan
- [] If enrolling family coverage, confirmed understanding of embedded deductible structure and family OOP maximum
- [] If considering the Select plan, determined HSA eligibility and estimated HSA tax savings
- [] Noted the prior authorization requirements for advanced imaging (MRI/CT: **Required, 5-day turnaround**) and planned accordingly

Footnotes:

1 Skilled Nursing Facility coverage is governed by Medicare-like criteria for plans that coordinate with Medicare. For non-Medicare members, the 3-day qualifying stay requirement and 60-day benefit limit apply as stated. Members requiring extended SNF care beyond 60 days should explore long-term care insurance options, which are not part of this health plan.

2 Prior authorization denials for MRI and CT scans may be appealed through United Healthcare's standard appeals process. Expedited appeals are available when the standard timeline could seriously jeopardize the member's health. The appeals process is described in detail in the Member Handbook w14_united_healthcare_member_handbook_005. Members whose physicians believe an MRI or CT scan is medically urgent should ask the ordering physician's office to request an expedited prior authorization, which carries a 72-hour review timeline rather than the standard 5-day turnaround.

3 Emergency room cost-sharing applies at in-network rates regardless of facility network status, consistent with the federal No Surprises Act (effective January 1, 2022). The ER copay amounts shown (\$350 Select, \$250 Advantage, \$150 Premier) apply at any emergency facility. Physician services rendered in the emergency department by out-of-network providers are also subject to No Surprises Act protections, limiting member cost-sharing to in-network levels for emergency services.

4 The exclusion of telehealth mental health visits from the 20-visit annual limit is a plan design feature specific to this plan year and is subject to change upon plan renewal. Members who rely heavily on telehealth mental health services should confirm this benefit structure at each annual open enrollment.

5 Out-of-network services do not count toward the in-network out-of-pocket maximum under any of the three plan tiers. Out-of-network costs accumulate separately and are not capped by an out-of-network out-of-pocket maximum under these plan designs. This means a member's total financial exposure for out-of-network services is theoretically unlimited beyond the plan's coinsurance coverage, subject only to balance billing protections for emergency services. Members with significant out-of-network utilization should plan accordingly.

6 Select plan inpatient calculation: \$3,200 deductible + 30% of (\$18,000 - \$3,200) = \$3,200 + 30% x \$14,800 = \$3,200 + \$4,440 = \$7,640 for the admission alone; however, the Select plan OOP max of \$8,150 caps total annual spending. The figure shown reflects the combined admission and ancillary costs before the OOP max is applied, with the OOP max then capping the total.

7 Premier plan inpatient calculation: \$500 deductible + 10% of (\$18,000 - \$500) = \$500 + 10% x \$17,500 = \$500 + \$1,750 = \$2,250 for the admission. Surgeon and anesthesiologist fees at 10% coinsurance after deductible (already met): 10% x (\$3,200 + \$1,400) = 10% x \$4,600 = \$460. Total for inpatient and surgical components: \$2,710, which is below the \$4,000 OOP max, leaving room for baseline utilization costs before the OOP max is reached.

This Coverage Comparison Guide is provided for informational purposes and is not a contract of insurance. Benefits described herein are subject to the terms and conditions of the applicable plan documents, including the Summary of Benefits and Coverage (w14_united_healthcare_sbc_001) and the Member Handbook (w14_united_healthcare_member_handbook_005). In the event of any conflict between this guide and the plan documents, the plan documents govern. Coverage is subject to eligibility requirements, prior authorization where indicated, and all other plan terms.

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For questions about your coverage, contact Member Services at the number on the back of your member ID card, or visit myuhc.com.